Fregoli Syndrome

Fregoli Syndrome is a rare psychiatric and neuropsychiatric condition that is classified among the delusional misidentification syndromes. It was first described by Courbon and Fail in 1927. The fundamental characteristic of the syndrome is the belief that different people in the individual's surroundings are actually the same familiar person and that this person appears before them by disguising themselves as different individuals. In other words, the individual may recognize that the people in front of them have different appearances; however, they believe that they are actually the same person. Therefore, Fregoli Syndrome is considered to be associated with an impairment in the process of distinguishing and identifying one person from other people.

In Fregoli Syndrome, an individual may identify multiple strangers or different people in their surroundings as a single familiar person. The belief that the familiar person is following them, appearing before them by taking on the appearance of different people, or disguising themselves may be observed. In some patients, this misidentification may not be limited to a single person but may involve multiple individuals. In this respect, Fregoli Syndrome is not simply a situation in which "one person is mistaken for another"; rather, there is a strong and firmly held false belief regarding the identity of the people in front of the individual.

Fregoli Syndrome may occur on its own or may be observed in association with various psychiatric or neurological conditions. Among 119 patients examined, it was reported to be associated with primary psychosis in 52% and secondary psychosis in 42% of cases. Schizophrenia was the most common diagnosis in the primary psychosis group, whereas neurological diseases, stroke, and traumatic brain injury were particularly prominent among secondary cases. Therefore, in an individual presenting with Fregoli Syndrome, not only psychiatric causes but also possible underlying neurological causes should be taken into consideration.

Disorders involving particularly the right hemisphere and frontal regions of the brain are thought to be important in the development of Fregoli Syndrome. In patients who underwent neuroimaging, right-sided brain lesions were found to be more common than left-sided lesions. Lesions of the right hemisphere, particularly the right frontal region, have been reported to be prominent in delusional misidentification syndromes. It is thought that dysfunction in these regions may affect processes such as familiarity, reality assessment, and the integration of memories, thereby contributing to misidentification.

Facial recognition and emotional evaluation processes are also important in understanding Fregoli Syndrome. It has been suggested that abnormal functioning of the fusiform face area in the ventral temporal region, which is associated with facial recognition, and the amygdala, which is associated with emotional regulation, may be related to delusional misidentifications. Fregoli Syndrome is considered a form of "hyper-identification," and it has been suggested that the individual's ability to distinguish the unique identity information specific to a particular person may be impaired.

Fregoli Syndrome may occur in some cases during the first psychotic episode. Patients with secondary psychosis have been reported to be more likely to experience Fregoli Syndrome during their first psychotic episode compared with those in the primary psychosis group. In addition, patients in the secondary psychosis group have been reported to be older. These findings demonstrate the importance of investigating underlying neurological causes, particularly in individuals who present with Fregoli symptoms at an advanced age or during their first psychotic episode.

In terms of diagnosis, there is no single laboratory test or imaging method that can definitively confirm Fregoli Syndrome. Therefore, a detailed psychiatric, neurological, and cognitive assessment is important. Brain imaging methods may be utilized when necessary. In terms of treatment, there is no single definitive standard for Fregoli Syndrome; the approach is determined according to the underlying cause and comorbid conditions. Antipsychotic medications may be used to control psychotic symptoms, and different treatment methods may be considered in some treatment-resistant cases.

In conclusion, Fregoli Syndrome is a rare delusional misidentification in which an individual perceives different people as different appearances of the same familiar person. It may be associated with psychiatric disorders such as schizophrenia, as well as occurring following neurological conditions such as stroke and traumatic brain injury. Findings particularly involving the right hemisphere and frontal regions highlight the neurological aspect of the syndrome. Therefore, Fregoli Syndrome should not be considered solely as a psychiatric symptom; when necessary, underlying neurological and cognitive causes should also be investigated.

References:

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